جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : محمد عصام شعبان عبد العظيم
المحافظة : بني سويف **الحي / المدينة :** سمسطا **العنوان :** /*/////
رقم البطاقة : 30410172200174 **رقم التليفون :** 01203469105 **تاريخ تسجيل الإستمارة :** 11-10-2026
العمر(تاريخ الميلاد) : 17-10-2004 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان الكبد
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان الكبد
خطة العلاج : أبحاث أورام مسح ذري علي الجسم [8530] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: محمد عصام شعبان عبد العظيم
National ID: 30410172200174

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

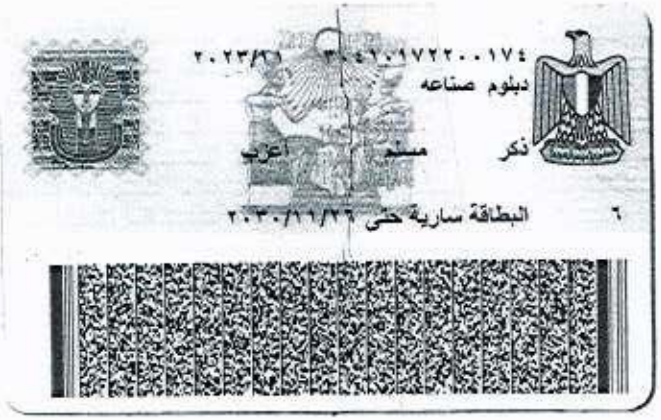
تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

A patient of liver cancer for Whole-body PET/CT scan

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Dr.
Elia Anis Ishak
M. Sc. Ph. D. Pathology - Prof. of Pathology , Cairo University
Fellow, Armed Forces Institute of Pathology , USA
Member, United States Canadian Academy of Pathology

Dr.
Shady Elia Anis
M. Sc. MD Pathology , Ass. Prof. of Pathology , Cairo University
Fellow , Mass General Hospital , Harvard , USA
Member , United States Canadian Academy of Pathology

Patient Name : محمد عصام شعبان عبد العظيم
Age : 19 Y : 6 M : 2 D
Referred By Prof. Dr. : طه ياسين
Clinical Diagnosis : Suprarenal mass
Nature of Specimen : Excision
Receiving Date : 2024-05-11

Analysis ID : 848049
Gender : Male

Patient ID : 639390

PATHOLOGY REPORT

Gross :

Well defined mass, measured 19x16.5x10 cm, weighed 1350 grams. Outer surface was smooth showed overstretched suprarenal gland tissue. The mass showed rubbery lobulated tan greyish and white hemorrhagic cut section with focal areas of necrosis. The mass was surrounded by thin capsule [inked blue], cauterized surgical margin [inked green].

Microscopic :

Sections examined from the specimen received revealed adrenal tumor tissue formed of large nesting pattern with zell ballen and surrounding sustenticular cells. The cells are polyhedral with ill defined border had eosinophilic to vacuolated cytoplasm, moderate pleomorphism Mitotic figures more than 3/10 HPF. Occasionally hyperchromatic nuclei.
There is foci of necrosis.
No capsular invasion.
No extension to fat.
No spindling.
No vascular invasion

Diagnosis :

Suprarenal mass, Excision, PHEOCHROMOCYTOMA PASS SCORE (10).
- CASE FOR CONFIRMATION BY IMMUNOPHENOTYPING.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

This report is electronically Signed , Any erasure or modification voids this report

DR ELIA ANIS LABS
عمارة رمسيس - بلوك ا - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٤٧٣٢ - ٢٥٧٧٣٢٥٣ - ٢٥٧٩٠٩٣٥ - ٠١٢٨٧٠٠٥٢١٧

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: محمد عصام شبعان عبدالعظيم **MR. :** 9131	

Detailed Medical Report

السيد	

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient is a 21-year-old individual with neuroblastoma (NB). Initial therapy consisted of four cycles of an unspecified regimen, followed by surgical excision, which marked the end of first-line treatment. A follow-up PET-CT performed in October 2025 showed an operative bed free of disease but progressive abdominal lymph nodes. Biopsy of the nodes confirmed NB ALK-positive disease, while bone marrow aspirate and biopsy were negative. Consequently, second-line therapy with the NB HR protocol was initiated. After the fifth cycle of this regimen, a PET-CT demonstrated morphological and metabolic regression of the metastatic abdominal lymph nodes (size reduced from 2.2 × 1.4 cm to 1 cm, SUV decreased from 3.3 to 2.5), with the operative bed remaining free and no new lesions identified. The patient is considered a candidate for autologous bone marrow transplantation (Auto-BMT). The first cycle of the Topotecan/Cyclophosphamide (Topo/Cyclo) component of the HR protocol was administered with the following details: Topotecan 1.32 mg IV, Cyclophosphamide 437.5 mg IV, Mesna 87.5 mg IV 3 hours after endoxan, hydration 1000 mL over 4 hours, and supportive care with Zofran 16 mg IV (days 1-5) and Sutrim 160 mg q12 h for 3 days per week. A second cycle is scheduled after a two-week interval to monitor laboratory parameters.

تحريرا في ,

التاريخ :	**21-09-2026**	**الوقت :**	

الطبيب المعالج : **Ahmed Mostafa Abdelsalam**

يعتمد ,

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Member , United States Canadian Academy of Pathology

Patient Name : محمد عصام شعبان عبد العظيم
Analysis ID : 850275
Age : 19 Y 7 M : 1 D
Gender : Male
Referred By Prof. Dr. : طه ياسين
Clinical Diagnosis : Suprarenal mass
Nature of Specimen : Blocks (Excision, labeled as 848040, Blocks A&B)
Receiving Date : 2024-05-18
Patient ID : 639390

IMMUNOHISTOCHEMISTRY REPORT

Technique :

Sections were prepared from the paraffin blocks, processed in Benchmark Ultra machine (Ventana) and stained by rabbit monoclonal antibodies using Ultra view HRP DAB as chromogen and hematoxylin as counter-stain .
The sections were treated against:
- Synaptophysin \ NSE (Neuron Specific Enolase) \ S 100 \ Ki67

Results :

- Synaptophysin showed POSITIVE tumor cells with positive ganglion cells.
- NSE (Neuron Specific Enolase) showed POSITIVE tumor cells with positive ganglion cells.
- S 100 showed no evidence of sustentacular cells
7 % of Tumor cells are POSITIVE for Ki67.

Conclusion :

Block, THE IMMUNOHISTIOCHEMICAL RESULT ARE MORE COMPATIBLE WITH GANGLIONEUROBLASTOMA.
N.B: THE PREVIOUS HISTOLOGIC IMPRESSION OF PHEOCHROMOCYTOMA IS EXCLUDED BY THE ABSCENCE OF SUSTENTACULAR CELLS.

Prof. Dr. Shady Elia Anis

Prof. Dr. Elia Anis Ishak

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عمارة رمسيس - بلوك أ - ميدان رمسيس - الدور السادس و السابع
تليفون : ٢٥٧٤٢٧٤٢ - ٢٥٧٧٢٢٥٣ - ٢٥٧٩٠٣٥ - ٠١٥٢١٧٠١٢٨٧

Immunohistochemistry
[[DOCTYPE:IHC]]

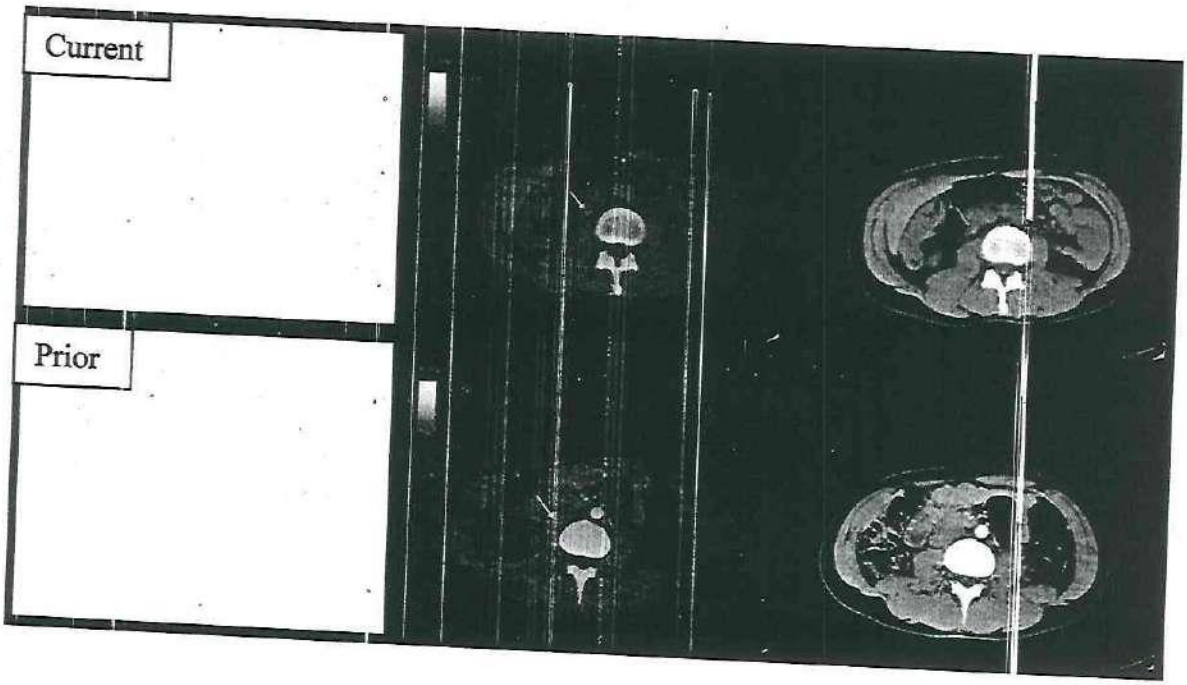
Whole Body FDG-PET/CT Scan

History and clinical findings:

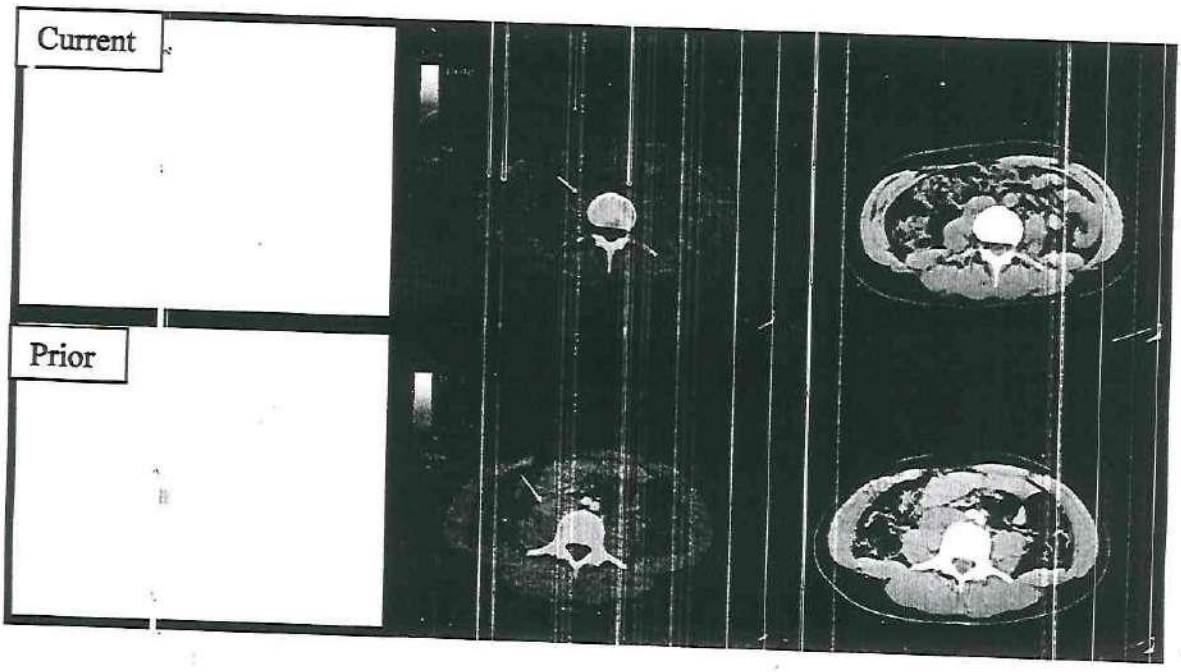
- 21-year-old male patient, known case of right supra-renal ganglioblastoma, underwent surgical excision then received chemotherapy, last cycle was 2 weeks ago, for therapy assessment and follow up.
- **Previous PET scan dated 2/11/2025 is compared.**

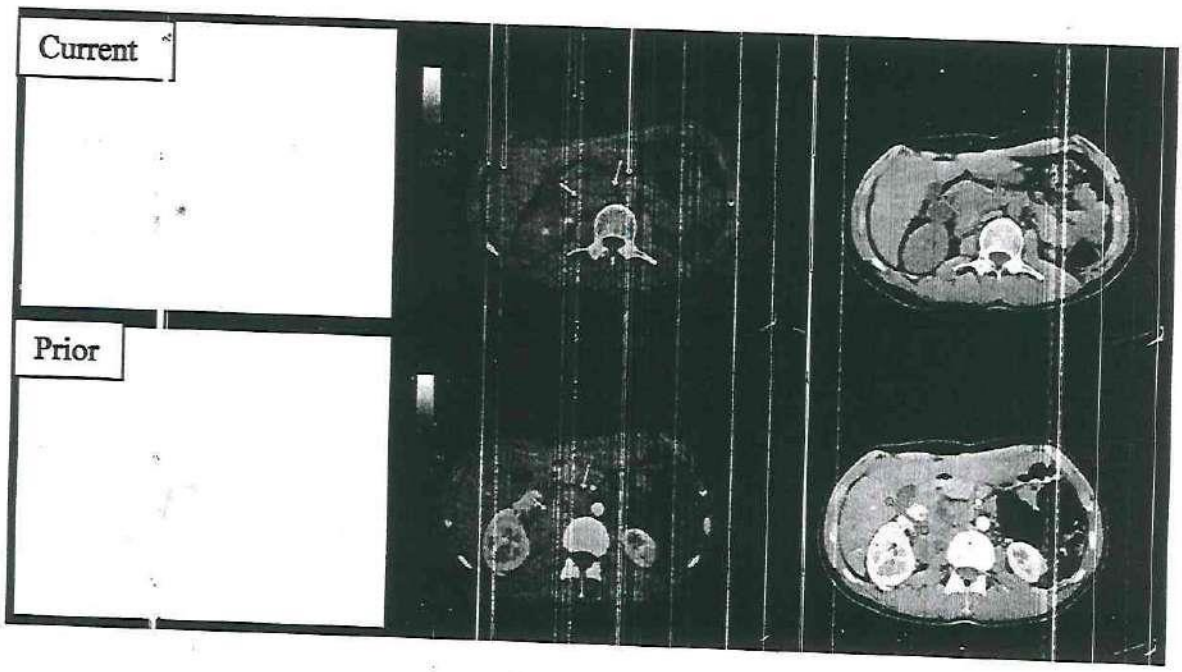
PET findings:

- **Right supra-renal:**
  - **Operative bed: status of mass excision with no FDG avid recurrent lesions.**
- **Lymph nodes:**
  - **Metabolic and morphologic regression of the previously noted FDG avid right peri-caval, right renal hilar, retro-caval and peri-pancreatic nodal lesions, currently representative retro-caval LN with SUVmax 2.5 at and measures about 1cm compared to SUVmax3.3 and 2.2x1.4cm previously.**
  - **No other FDG avid or pathologically enlarged lymph nodes along the rest of scanned body.**



PET Scan
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- **Whole body:**
  - The rest of the scanned body regions show no increased pathological FDG activity that could account for other metastatic sites.

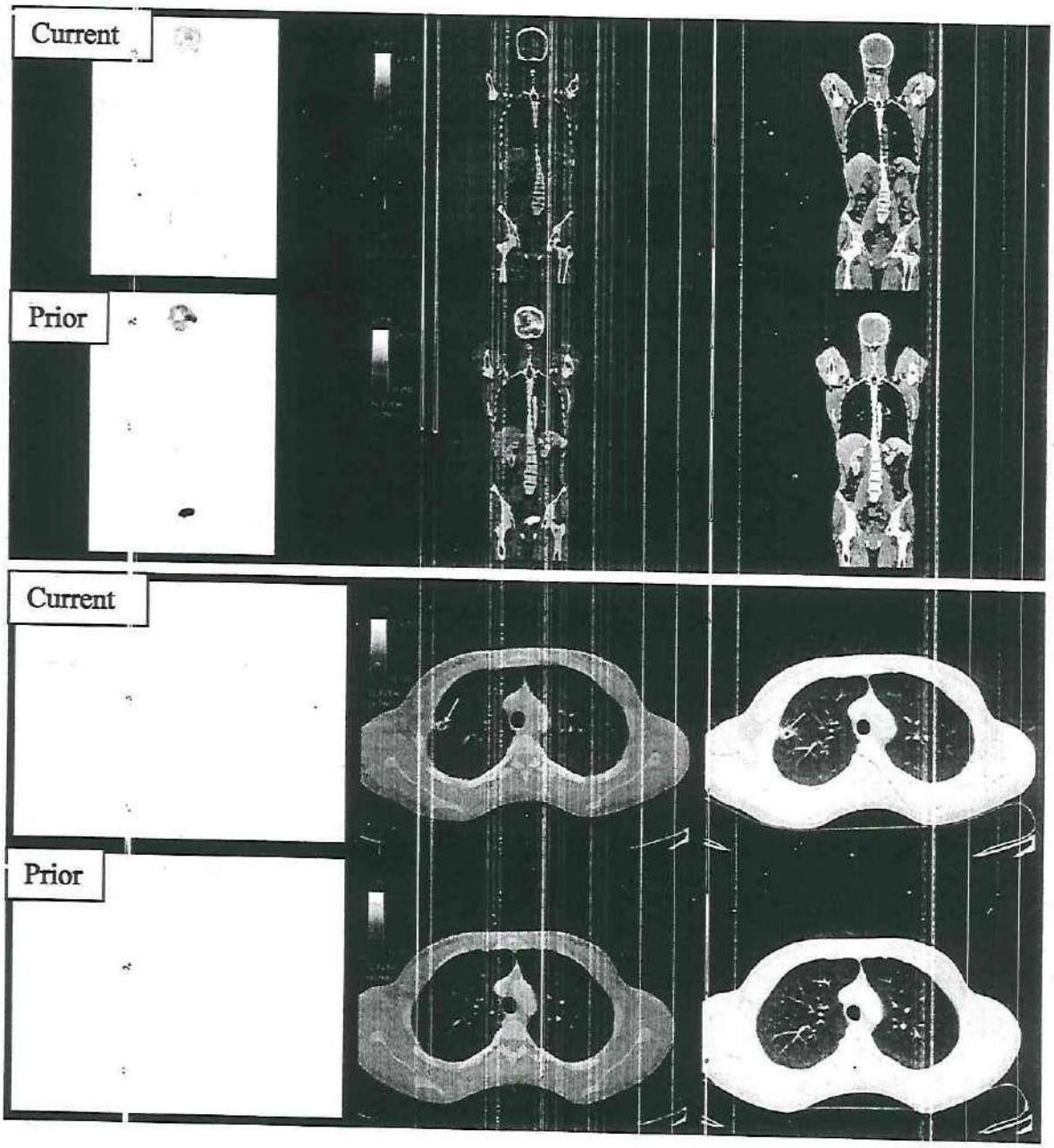
PET Scan
[[DOCTYPE:PET_SCAN]]

CT diagnostic findings:

- **Head and neck:**
  - Normal appearance of the naso, oro- and hypopharynx with preserved para-pharyngeal fat planes.
  - Intact cartilagenous skeleton of the larynx with no glottic, supra- or infra-glottic lesions detected.
  - Normal appearance of the submandibular and parotid glands.
  - Normal appearance of thyroid gland.
- **Chest:**
  - No evidence of pleural effusion, pleural thickening or masses.
  - Normal appearance of cardiac chambers.
- **Abdomen and pelvis:**
  - Average sized liver showing homogenous parenchyma, no FDG avid focal lesions.
  - Average sized spleen with no focal lesions.
  - Normal appearance of kidneys, normal cortical thickening, no stones or backpressure changes.
  - Normal appearance of pancreas, left supra-renal, aorta and IVC.
  - No abdomino-pelvic collection seen.
- **Musculoskeletal:**
  - No FDG avid bone deposits.

PET Scan
[[DOCTYPE:PET_SCAN]]

- **Interval development of mild FDG avid right upper lobe reticulonodular opacity with SUVmax3.4, likely inflammatory.**



PET Scan
[[DOCTYPE:PET_SCAN]]

Patient Name: Mohamed Esam Shaaban

Date: 25-4-2026

Referred by: Prof Dr. Mohamed Hammad

Whole Body FDG-PET/CT Scan

Conclusion

Post chemotherapy FDG-PET scan for right supra-renal ganglioblastoma showing:

- Status of mass excision with no evidence of metabolically active recurrent lesions.
- Metabolic and morphologic regression of the previously noted metastatic abdominal nodal lesions.
- No evidence of newly developed metabolically active metastatic lesions.
- Interval development of mild metabolically active right pulmonary reticulonodular opacity, likely inflammatory for clinical correlation and follow up.

Much Obliged,

Waleed Ahmed Diab, MD Assiut University	**Mahmoud Shaban Abd Al-Alim, MD,** EBNM European Board of Nuclear Medicine

PET Scan
[[DOCTYPE:PET_SCAN]]